

DUTCH
METHODS
OF
BIRTH CONTROL.

BY
MARGARET H. SANGER.

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METHODS OF BIRTH CONTROL ***

**DUTCH METHODS
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MARGARET H. SANGER.

DUTCH METHODS OF BIRTH CONTROL.

The following methods are taken from the pamphlet published by the Neo-Malthusian League of Holland, called "Methods Used to Prevent Large Families," translated into English from the Dutch.

The Council of the Neo-Malthusian League calls attention to the fact that it has for its sole object the Prevention of Conception, and not the causing of abortion.

The Neo-Malthusian League of Holland knows nothing of this pamphlet, and is not in any way responsible for its publication.

M. H. S.

A BRIEF HISTORY OF THE LEAGUE.

In the year 1877 Mrs. Annie Besant and Mr. Charles Bradlaugh, two firm and honest advocates of the doctrine of Malthus, were prosecuted and sentenced to imprisonment for publishing a book entitled "The Fruits of Philosophy," which presented the physiological aspects of birth control.

The trial lasted several days, and aroused a greater interest in the subject than had been known since the days of Malthus. The English Press was full of the subject; scientific congresses gave it their attention; many noted political economists wrote about it; over a hundred petitions were presented to Parliament requesting the freedom of open discussion; meetings of thousands of persons were held in all the large cities; and as a result, a strong Neo-Malthusian League was formed in London.

Interest in the subject did not confine itself to England, however, for the following year at an International Medical Congress in Amsterdam the subject was discussed with great enthusiasm. A paper prepared and read by Mr. S. Van Houten (later Prime Minister) caused a wider interest in the subject, and a year later the Neo-Malthusian League of Holland was organised. Charles R. Drysdale, then President of the English League, attended the Conference.

As is usual in such causes, many of the better educated and intelligent classes adopted the practice at once, as did the better educated workers; but the movement had as yet

no interest among the poorest and most ignorant. The League set to work at once to double its efforts in these quarters. Dr. Aletta Jacobs, the first woman physician in Holland, became a member of the League, and established a clinic where she gave information on the means of prevention of conception free to all poor women who applied for it.

Naturally, this action on the part of a member of the medical profession aroused the animosity of many of its members against her; but Dr. Jacobs stood firm in her principles, and continued to spread the necessary information among the peasant women in Holland in the face of professional criticism and gross misunderstandings.

All classes, especially the poor, welcomed the knowledge with open arms, and requests came thick and fast for the League's assistance to obtain the necessary appliances free of charge. The consequence has been that for the past twelve years the League has labored chiefly among the people of the poorest districts. Dr. J. Rutgers and Madame Hoitsema Rutgers, two other ardent advocates of these principles, have devoted their lives to this work. Dr. Rutgers says that where this knowledge is taught there is a reciprocal action to be observed: "In families where children are carefully procreated, they are reared carefully; and where they are reared carefully, they are carefully procreated."

The Neo-Malthusian League of Holland has over 5,000 men and women in its membership, and more than fifty nurses whom it endorses.

THE DISTRICT NURSES.

These nurses are trained and instructed by Dr. Rutgers in the proper means and hygienic principles of the methods of Family Limitation. They are established in practice in the various towns and cities throughout Holland. They not only advise women as to the best method to employ to prevent conception, but they also supply them with a well-fitted pessary, and teach them how to adjust, remove, and care for it—all for the small sum of 1½ guilder, or about 60 cents. They work mainly in the agricultural and industrial districts, or are located near them; and their teachings include not only the method of prevention of conception, but instruction in general and sexual hygiene, cleanliness, the uselessness of drugs, and the non-necessity of abortions.

In this country, for a nurse to dare to fit a woman with a pessary would be considered a breach of professional rights. In Holland, they know the poor cannot pay the physicians, and this simple adjustment is looked upon by the medical profession much as they view the nurse administering an enema or a douche.

I had the pleasure of attending some of the classes where Dr. Rutgers gave this course of instruction. I also attended and assisted in the clinics where women came to be advised, instructed, and fitted. Many of them came for the first time, and though they were unacquainted with any means to be employed, they accepted the instructions in a most natural and intelligent manner. Other women came to be refitted, and many brought the pessary previously used, to ask questions concerning its adjustment.

There was a determined social responsibility in the attitude of these peasant women coming into The Hague

from the surrounding districts. It seemed like a great awakening. They look upon a new baby in the family much as they look upon the purchase of an automobile or a piano or any other luxury where they have no room to keep it, and no means with which to purchase it or to continue in its upkeep.

There is no doubt that the establishment of these clinics is one of the most important parts in the work of a Birth Control League. The written word and written directions are very good, but the fact remains that even the best educated women have very limited knowledge of the construction of their generative organs or their physiology. What, then, can be expected of the less educated women, who have had less advantages and opportunities? It is consequently most desirable that there be practical teaching of the methods to be recommended, and women taught the physiology of their sex organs by those equipped with the knowledge and capable of teaching it.

The Neo-Malthusian League of Holland endorses, as the most reliable means of prevention of conception, the Mensinger pessary (which differs in construction from the French or the American Mizpah pessary). The nurses also recommend this; but other methods are discussed with the patient, and the husband's attitude toward other methods considered and discussed. The pessary is the commonest recommendation, as giving the most satisfactory results.

RESULTS.

It stands to the credit of Holland that it is perhaps the only country where the advocates of Birth Control have not been prosecuted or jailed. This does not mean there has been no opposition to this propaganda; on the contrary, there is to-day strong opposition by the Church, and only a few years ago, in 1911, when a Clerical Government came into power, laws were made against the propagation of these ideas, and much of the freedom previously enjoyed by the League was denied it; but on the expulsion of the Clerical Government later on, the rights of free speech and free press were regained.

In the year 1895 the League was given a royal decree of public utility, which again does not necessarily mean this propaganda is sanctioned by the Government; but the laws regarding the liberty of the individual and the freedom of the press uphold it, and it is thus that its advocates are not molested.

The League has thirty sub-divisions, which include all the large cities and towns and many of the smaller ones. Over 7,000 pamphlets of the kind printed herein are sent out in the Dutch language and several hundred in English and Esperanto each year. These are only sent when asked for by the applicant.

There is no doubt that the Neo-Malthusian League of Holland stands as the foremost in the world in organisation, and also as a practical example of the results

to be obtained from Birth Control teaching. Aside from the spreading influence of these ideas in Belgium, Italy, and Germany, Holland presents to the world a statistical record which proves unmistakeably what the advocates of Birth Control have claimed for it.

The infantile mortality of Amsterdam and The Hague is the lowest of any cities in the world, while the general death rate and infantile mortality of Holland has fallen to be the lowest of any country in Europe. These statistics also refute the wild sayings of those who shout against Birth Control and claim it means race suicide. On the contrary, Holland proves that the practice of anti-conceptional methods leads to race improvement, for the increase of population has accelerated as the death rate has fallen. There has also been a rapid improvement in the general physique and health of the Dutch people, while that of the high birth rate countries, Russia and Germany, is said to be rapidly deteriorating.

The following figures will suffice to show some of the improvements which have been going on in Holland since 1881, the time the League became actively engaged in the work:—

VITAL STATISTICS OF CHIEF DUTCH TOWNS.

Taken from Annual Summary of Marriages, Births, and Deaths in England and Wales, etc., for 1912.

AMSTERDAM (Malthusian League started 1881; Dr. Aletta Jacobs gave advice to poor women, 1885).

	1881- 85.	1906- 10.	1912.	
Birth Rate	37·1	24·7	23·3	per 1,000 of population.
Death Rate	25·1	13·1	11·2 ^[1]	" "
Infantile Mortality (deaths in first year)	203	90	64 ^[1]	per 1,000 living births.

THE HAGUE (now the headquarters of the Neo-Malthusian League).

	1881- 85.	1906- 10.	1912.	
Birth Rate	38·7	27·5	23·6	per 1,000 of population.
Death Rate	23·3	13·2	10·9 ^[1]	" "
Infantile Mortality (deaths in first year)	214	99	66 ^[1]	per 1,000 living births.

ROTTERDAM.

	1881- 85.	1906- 10.	1912.	
Birth Rate	37·4	32·0	29·0	per 1,000 of population.
Death Rate	24·2	13·4	11·3	" "
Infantile Mortality	209	105	79	per 1,000 living births.

¹. These figures are the lowest in the whole list of death rates and infantile mortalities in the summary of births and deaths in cities in this Report.

FERTILITY AND ILLEGITIMACY RATES.

	1880- 2.	1890- 2.	1900- 2.	
Legitimate Fertility	306·4	296·5	252·7	Legitimate Births per 1,000 Married Women aged 15 to 45.
Illegitimate Fertility	16·1	16·3	11·3	Illegitimate Births per 1,000 Unmarried Women aged 15 to 45.

THE HAGUE.

	1880-2.	1890-2.	1900-2.
Legitimate Fertility	346·5	303·9	255·0
Illegitimate Fertility	13·4	13·6	7·7 ^[2]

ROTTERDAM.

	1880-2.	1890-2.	1900-2.
Legitimate Fertility	331·4	312·0	299·0
Illegitimate Fertility	17·4	16·5	13·1

². Lowest figure for the Continent.

The Neo-Malthusian propaganda in Holland is founded on feministic and economic grounds. Birth Control is the pivot around which both of these important movements must swing. Mr. S. Van Houten, late Minister of the Interior of the Netherlands, says: "Wage-slavery exists as a consequence of the carelessness with which the former generation produced wage-slaves; and this slavery will continue so long as the adult children of these wage-slaves have nothing better to do than to reproduce wage-slaves."

There has been marked improvement in the labor conditions in Holland during these last ten years especially,

wages having increased and hours of labor decreased, with the cost of living taking a comparatively very small rise.

There is perhaps no country in Europe where the educational advantages are so great as in Holland. Many English women and widows with children, living on small incomes, move to Holland on account of the splendid education to be obtained in the public schools.

That the Birth Control propaganda has been a success in Holland any one travelling through that delightful, clean, and cheerful country can testify. There is no doubt that at first there were objections and abuses, but, in the words of Dr. Rutgers, "Certainly there are abuses, but the abuses of knowledge are never so enormous as the abuses of ignorance. And hygiene is the highest form of morality."

Following are the methods, reprinted from the practical pamphlet given out by the Neo-Malthusian League of Holland:—

Confidential.

MEDICAL INFORMATION.

Considerations of health, and uncertainty as to the rearing of her children, or any other personal cause may urge the mother of a family to avoid frequent births. For this purpose precautions are required by the married partners; and these means may be employed by the wife as well as by the husband. These methods are not absolutely infallible. (The only infallible methods are operations: viz., the obstructing by a ligature or by compression of the Fallopian tubes; the extirpation of the uterus or the ovaries in the female, and of the testes in the male.) They must be made use of very carefully and perseveringly if the couple wish to reduce to a minimum the chances of increasing the number of children. We shall give the minutest details about each of the methods known at the present day, insisting, last of all, upon the *injections*, for the essential part of every preventive method consists in cleanliness.

It is in the case of women who have had children that the methods about which we have to speak are most easily applied; for the narrower the female passage, the greater is the difficulty in making use of them. Of course, we shall not speak here of sexual satisfaction such as masturbation, which may be dangerous to health, consisting in an artificial excitation rather than in the appeasing of a physiological need.

When the accoucheur or midwife who examines a woman in labour pushes the finger, moistened with a little soap, as far as possible into the vagina, a small protuberance is felt like the tip of the finger, which is the lower end of the womb. The womb, the mouth of which is felt with some difficulty in the centre of the protuberance, is the little nest in which the human egg is hatched.

As soon as the male fluid (sperm) has arrived within the orifice of the womb the woman can no longer prevent impregnation, even if she immediately use vaginal injections with the most sedulous care; and to inject fluid into the womb or to make any other attempt to destroy the ovulum within the womb is more or less dangerous, as it may produce fever or hæmorrhage. These operations are in the department of medicine or surgery, and are only permissible for the prevention of some dangerous disease, or danger of sudden death. The medicines or poisons recommended for re-establishing menstruation are rarely of any effect, and are dangerous to life.

The following methods have simply the aim of preventing the male fluid from entering into the mouth of the womb; they in nowise injure the health, and are not forbidden in any civilised country either by moral codes or by law.

MEANS TO BE EMPLOYED BY THE HUSBAND.

The principal anti-conceptual method is, of course, *absolute continence* maintained for a long period, and this is needed in some cases, when the wife is ill, for instance, or when she requires perfect repose. The wife who wishes to remain free by this means ought nevertheless to take great care as to cleanliness, and if the husband forget himself for a moment, she should immediately give herself energetic injections; but this may be too late.

Some physicians think that *periodical abstention* during menstruation and for a week before and after the period would suffice to prevent conception; but this cannot be trusted to. It is true that women are less apt to be impregnated at these times; but cases have been known of numbers of pregnancies taking place in spite of this precaution.

During the period of lactation the chances of pregnancy also are less; but there is no certainty in this case.

By the effort of the will and by practice, it is possible for certain men to practice connection frequently without expulsion of the male fluid; just as it is possible for civilised persons who are intelligent to keep back their tears even when deeply moved. This kind of special faculty of prevention has been called *karezza* (to be dealt with in full in the next pamphlet).

When the husband can employ no other method, he can always practise *withdrawal* out of the female passage before emitting; but it might suffice for the male fluid to moisten the exterior genital parts of the wife to cause impregnation.

This method is simple, without expense and requiring no preliminary preparation, but the wife should inject herself immediately with care if she suspects any negligence on the part of the husband. He too ought to wash himself thoroughly (pushing back the prepuce) before recommencing relations with his wife within 24 hours.

When the effort is not too violent for the man, and if the wife is more or less satisfied, this method is not injurious to the health. At any rate occasionally another method may be used.

When the male organ is covered by an indiarubber *French Letter*, condom, or Malthus sheath, a little bag with thin walls, there is no chance of the woman being impregnated if the bag be not torn. In all instrument-makers' shops they are sold at from two to three shillings the dozen. One rather too large is to be preferred, for these bags contract if used more than once.

When the husband makes use of the French Letter, it must be unrolled, pushing it up not quite to the end, expressing with the hand the air bubbles which may remain in its cavity; the bottom may also be moistened inside with a little soapy water or saliva.

If, after using it, the French Letter be empty, it is evident that it must have been torn, in which case the wife should immediately use careful injections; but this may be too late.

One letter may suffice for several times; in this case, it must each time be carefully wiped off or washed and dried and inflated in order to remove the folds, and to see that it has not been torn; and lastly, it may be powdered with a little powder inside and outside. The best powder is that of lycopodium, which is obtainable at a cheap rate in druggist shops. After having powdered it, the French Letter may be opened by means of two fingers, and be rolled up again when required for further use.

The same French Letter should not be too often used, perhaps two weeks or six times, and it should not be too old, for in the lapse of time it loses its properties. Like all things made of indiarubber, it is well to keep it in a rather damp, cool place, and shield it from light or frost, it must not be touched by greasy substances, such as oil, fat, vaseline, paraffin, etc., nor with carbolic acid or other substances which act upon indiarubber.

There are also French Letters or condoms of gut, called *Skin Letters*, made from the cœcum of sheep. These are sold at higher prices, and are more durable and stronger than the indiarubber ones. They have the fault that they become somewhat hard after frequent use. They are not elastic, and therefore cannot be rolled up, so that they cannot be used if they are too narrow. Before use, it should be carefully examined against the light to see that it is not made of pieces glued together, which fall asunder as soon as the bag is moistened, and then it must be blown up gently to see that there are no holes in it. When good, it may be used for a month. Their employment is quite like that of indiarubber letters.

The French Letter is the only preventive which diminishes a great deal the chances of contracting venereal diseases.

When there is any chance of venereal contagion, it is necessary to wash the French Letter with a solution of corrosive sublimate (perchloride of mercury), one in 10,000 parts of water; then it should be dried and powdered. By using this solution immediately after connection, it is seen at once whether the French Letter has been torn; in this case, the man must wash himself, and the woman should use an injection with the same solution, not only to prevent contagion to both of themselves, but she also to prevent the birth of an infected infant.

When the husband is drunk, and his wife, fearing that a miserable child will be born, has no other preventive at hand, she can perhaps apply the French Letter as if caressing him, when he does not know what he is doing. At all events, she should always take care that one or two French Letters be ready for use.

MEANS THAT MAY BE USED BY THE WIFE.

The wife may prevent conception by passing into the vagina the *occlusive pessary* of *Dr. Mensinga*; if this do not remain in its place, she may rather use that named *Matrisalus*, which is more curved. If this also does not remain in its place, the husband should use the French Letter or condom. If, finally, the husband will not use these means, the wife may use the *tubular pessary*, or, for want of a better, the *sponge*. (This is the only method available in countries where there are no medical practitioners or other persons who know the business, and can choose and teach women how to place the pessaries.)

These instruments, designed to cover the mouth of the womb, are not worn in the daytime. The wife may introduce them every evening (and by preference before the return of her husband). During the night they all may remain untouched, unless there be danger of the instrument being ill-placed. On the morrow, or in uncertain cases immediately after connection, a small injection should first be made, in order to cleanse away the greater part of the sperm, then the instrument should be withdrawn standing upright or kneeling; finally a full injection should be made use of to clear out thoroughly every corner of the organ. The pessary must then be cleaned carefully, and inspected to see that it is in good condition, wiped gently and put away in a drawer without wrapping it up.

During menstruation the woman should entirely abstain from sexual relations. When, however, it should chance that, withdrawing the instrument she sees that the flow appears, it will be necessary for her to use energetic injections if she had sexual relation.

The method which we mention for the use of women has the great advantage of permitting her to be free from care during the night; and it is also an essential point that the husband need not be consulted in the matter.

If these instruments are well placed, the husband cannot perceive their employment by the wife. It is also requisite that the method does not at all annoy the wife; if they produce the slightest pain, it is because they are either ill-chosen or misplaced.

Should the wife have any special disease of her organs, she should refrain from all connection until cure is effected; when there is doubt, she ought to consult a physician before employing such methods.

The first instruction and choice of the instrument should be pointed out to the wife by a practitioner or by a midwife or by any other person acquainted with the matter. If such assistance be unobtainable, she may herself try to act under the following directions:—

The *pessary Mensinga* (price 50 cents) is a simple ring closed by an indiarubber membrane curved like a hemisphere; it is but of little importance whether the convex side be directed upwards or downwards. These instruments are made of different sizes, and are numbered corresponding to

their diameters in centimeters. The greatest number that can be introduced without discomfort gives the most security. (We advise engaged women to choose their pessary some weeks before marriage, in order that the discomfort which results for the first moment may have passed away on the marriage day; $6\frac{3}{4}$ or 7 will generally be suitable; soon after marriage they should choose higher numbers. The external orifice of the genitals may be very narrow, and yet the vagina rather large). It is good to commence by trying No. $7\frac{3}{4}$.

Generally the pessary is moistened with the same liquid which is used as an injection; but on the first occasion, and always when the pessary is introduced with difficulty, the genital parts may be moistened with white soap to render them slippery. When a wife is measured for a pessary she should be at her ease, undressed, without her stays, in the stooping or cowering posture, and thighs apart. She should have been to the watercloset before.

To place the pessary, it is pushed vertically into the longitudinal opening of the vulva. The ring may be slightly pressed into a figure of 8, but not forcibly so as to break the spring which is in its walls. The part first introduced ought to be directed backwards; the last inserted part should disappear behind the os pubis which is felt in front. By hooking the finger behind this bone, the pessary may be pushed up as far as it will go.

It is necessary to choose the pessary with care. The essential conditions are: (1) that there should be no space between the pessary and the os pubis, nor that any should remain when it is pushed as far as possible backwards. (A very small space may perhaps be left, on condition that the husband take care not to enlarge it during connection.) (2) We should feel the inferior tip of the womb covered by the membrane of the pessary when the finger is introduced as far as possible. As a rule, we should try the largest number that can enter, and then higher numbers, until the pessary is found which satisfies these conditions.

If the membrane of the pessary does not cover the mouth of the womb, the pessary must be introduced not in the direction of the abdomen, but backwards towards the anus (bottom); this method of operating may fail if the woman is stooping; it will succeed better if she is lying down.

When a pessary is found which realises the conditions above described, care must be taken that the instrument remains in its place when the woman is standing up with the thighs apart, and making at the same time pressure of the abdomen. If in such conditions the pessary descends so that a space is left between the pessary and the os pubis, a larger number must be tried. If no number of the Mensinga pessary fits, recourse must be had to the *pessary Matrisalus* (price 75 cents), which is more difficult of application, for with this instrument care must be taken that the convex side is placed above, and the curved part in front, in correspondence with the curved part of the os pubis. This pessary has the great advantage that it does not descend so easily. Besides, this pessary is treated like the others.

If this pessary does not suit either, the husband must make use of the French Letter, and if he objects, the wife might use the *tubular pessary* (price 25 cents), which, not serving as an obturator of the vagina, covers the end of the womb like a cap. The wife at first introduces her finger to find out the tip of the

womb, she then slides the tubular pessary up until it adheres like a sucker to its tip. The higher part of the pessary should enter first, and the instrument should be placed so that during connection the husband should scarcely touch the bottom of the pessary. After the first connection, the wife should see that the pessary remained in its place.

Injections should be used as in former cases. If none of these methods are applicable, the wife may make use of a fine *sponge*. This sponge ought to be as large as an infant's fist, and be rather too large than too small, so as to block up the vagina.

A thin ribbon is attached to the sponge, in order to withdraw it easily. The sponge must be renewed after a time, as it loses its elasticity with use. It should, before it is introduced, be moistened with the same solution as the fluid injection, and then pushed up so that the womb is well covered. Injections should be used with the sponge before it is introduced and immediately after connection, before and after removing the sponge. It is not as certain a preventive as the pessaries.

Yet this sponge is preferable to certain plans spoken of in the newspapers, and which cost a good deal, such as *soluble pessaries*, containing quinine or some acid substance. These are pushed up five or ten minutes before connection, and as near as possible to the mouth of the womb, with the hope that they may melt at the right moment and at the right place to destroy the vitality of the male fluid.

The *Atokos* or other syringes with powder. They contain an acid powder which is blown up into the vagina. If either of these methods be used, energetic injections should be taken immediately after connection.

Injections, made immediately after connection, even with the most splendid syringes, are not of themselves sufficient; they always can come too late.

Dr. HINZ has invented a small syringe with an indiarubber ball at each end of a tube, recommended by Dr. FISCHER-DUCKELMANN, to inject a spermicide liquid, which is warmed, at the moment of the emission. This syringe is called *Facilitas*, but it is neither easy nor secure.

With these syringes the liquids mentioned in the following chapter may also be used.

For the success of any of these methods, it is indispensable that the wife should be acquainted with the position of the mouth of the womb.

INJECTIONS.

Injectons are an essential point of the sexual hygiene of the wife; but they are not sufficient alone: they complete the other preventive methods.

As injection, any acid solution may be used; for instance, *vinegar* with equal parts of water, or a solution of 1 per cent. of citric or tartaric acid, etc. Any astringent solution, which is also useful in the case of white discharges: *sulphate of zinc* or *alum*, of either 1 per cent. (a dessertspoonful of the powder in a litre or in a large bottle of water). Or *corrosive sublimate* (perchloride of mercury), a decigramme dissolved in a large bottle of water.

This last solution is also very powerful against venereal contagion; but, if too frequently employed, it may prove poisonous.

Solutions of copper or nitrate of silver may be used; but these solutions stain the linen. Copper, if long used, is poisonous.

Now, what syringe should be used?

The simplest instrument is the *glass syringe*, not curved, of large size (containing 60 cmc. of liquid), which costs 25 cents. The solution is poured into a cup and drawn up into the syringe; the piston should be so carefully fitted that no fluid will escape when the syringe is held downward. The most efficacious method of using the injection is to lie on the back, on a vessel receiving the fluid, the thighs drawn up and separated. The syringe is pushed up into the vagina as far as possible, and then this piston is rapidly pushed down; when this has been done, the syringe is moved from right to left, in order to wash out all the folds of the vagina.

Other instruments may as well be employed if it be only a syringe with a long tube, that it may reach all vaginal folds—the *clysopompe*, which acts by means of a spring; the *clysior* or oblong bulb, with a tube at each end; the *irrigator*, which is hung up on a nail high up on the wall.

These latter instruments require a much greater quantity of the solution than the glass syringe requires. Therefore the fluid, almost a litre, should be warmed when the weather is very cold, or when the woman is very sensitive.

The tube ought not to be too curved, and ought to be pushed up as far as possible and to be energetically moved in all directions, so as to be sure that nothing remains in the vaginal folds.

The least useful syringes are the indiarubber pears, by means of which it often happens that nothing more than the air contained in the pear is introduced into the vagina.

Two important Books by MARGARET H. SANGER—**What Every Girl Should Know**, price 25 and 50 cents, paper and cloth bound; **What Every Mother Should Know**,

paper cover, price 25 cents.—Address, M. MAISEL, 422
Grand Street, New York City.

TRANSCRIBER'S NOTES

1. Silently corrected typographical errors and variations in spelling.
2. Archaic, non-standard, and uncertain spellings retained as printed.

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